National Cancer Control Programme (NCCP) 00348 · clinician draft

carboplatin AUC 6 / paclitaxel → dose-dense AC

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Phase 2 · dose-dense AC

Active medicines: Doxorubicin + Cyclophosphamide

Emetogenic risk: High · AC

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 No routine Days 2–4 dexamethasone in this NCCP AC antiemetic row
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, interactions and glucose
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications

Additional phase-specific support

- Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) support is required for every dose-dense AC cycle; the team prescribes its product and timing separately.
- G-CSF every dose-dense AC cycle is required. Product, start day and duration must be prescribed using the local supportive-care policy; do not administer simultaneously with cytotoxic chemotherapy.
- Confirm cumulative doxorubicin exposure, cardiac assessment and venous-access plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00348 v8a · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-20 · SACTCheck v0.84.13 · Regimen-level drafts; prescriber and oncology-pharmacy verification required